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cine dose being administered in India. The world's fastest vaccination drive took nine months and, at its peak, 25 million (mostly free) doses were being given in 70,000 centres. More doses were given in rural India—a feat made possible by door-to-door campaigns and networks of field workers.

The fight against non-communicable diseases such as cancer, stroke and diabetes has received an infrastructure boost, with 640 district clinics and over 5,000 clinics at community healthcare centres. In addition, 194 cardiac care units (CCUs) and 239 day care cancer centres have also been set up across the country.

The MoHFW has made the National Digital Health Mission a priority, funded it handsomely and has brought in schemes like the hospital information system, the Nikshay-TB programme and Mera Aspataal, a patient feedback system. India also launched its tele-medicine programme, eSanjeevani, connecting 150,000 health and wellness centres to patients. During the pandemic, 15 million consultations were done through it. To address the burden of mental health disorders, the government has put aside funds to start a National Tele-Mental Health Programme—23 dedicated centres will provide free tele-counselling.

The urban poor have little access to health facilities, and the National Urban Health Mission is setting up a network of primary and community health centres to cater to their needs. Already, out of pocket expenditure (OOPE) for healthcare (expenditure borne directly by a patient) in India has declined from 64.2 per cent in 2014 to 48.8 per cent in 2018. Experts say that the increase in the budget's healthcare allocation to 2.5 per cent of the GDP would further reduce OOPE.

Furthermore, in recent years, India has witnessed a notable decline in the Maternal Mortality Ratio as well as the Neonatal Mortality Rate.

However, there are challenges ahead, and many have been doggedly persistent for decades: a less than ideal number of doctors and trained nurses, and the need for more hospital beds.

Keeping this in mind, there is a push to direct health resources where the disease burden is the greatest—the urban poor, village and block levels and remote areas— through a mixture of technology, field visits, improved community health infrastructure and setting up district-level medical colleges. The crucial goal here is to make healthcare accessible. Accessibility is healthcare's golden ratio now. ■